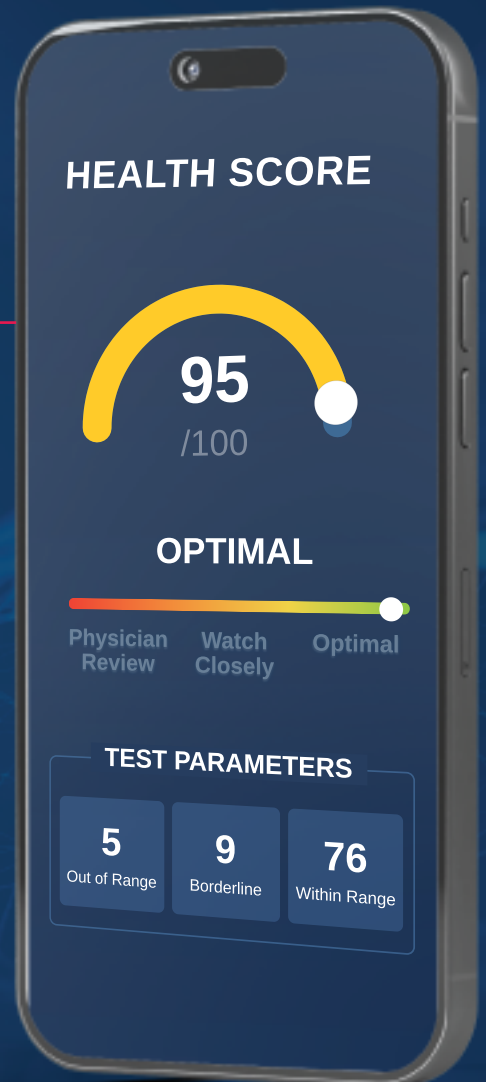


SMART HEALTH REPORT

An insightful Health Analytics Report
For Easier Understanding

Prepared For
**Ms Dr Namrata
Shrivastava**
Female 51 yrs



Trusted by Millions Across India

Collected By:
Certified & Trained
Professionals

Processed At:
RRL JABALPUR
NABL-Compliant Facility

Approved By:
Dr. Manju Saraf
MD, Pathologists



Ms Dr Namrata Shrivastava

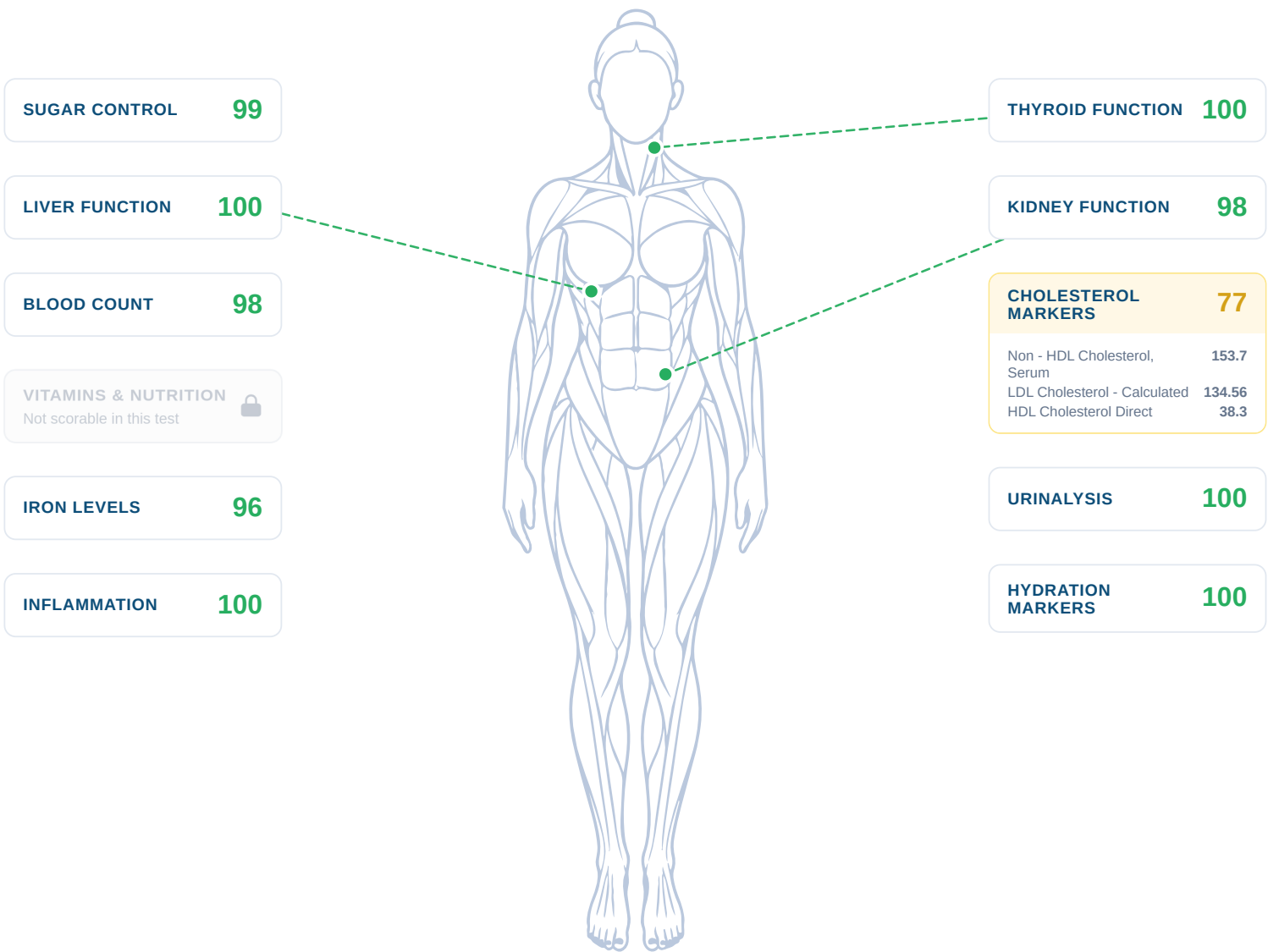
Gender: Female Age: 51 Yrs Patient ID: 16996029

Your Health Summary

Understand your health at a glance your personalised health summary is now available

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COMPREHENSIVE WELLNESS OVERVIEW



0-39

Physician Review

40-59

High Concern

60-69

Needs Action

70-79

Watch Closely

80-89

Monitor

90-100

Optimal

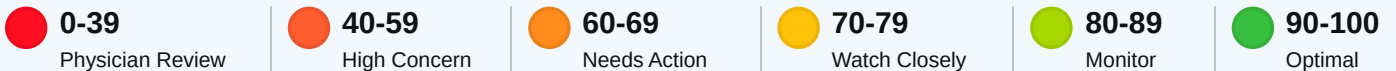
All domain scores are measured on a scale of 0 to 100, where a higher score indicates better health status

**Ms Dr Namrata Shrivastava**

Gender: Female Age: 51 Yrs Patient ID: 16996029

Your Health Summary

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[View detailed summary on our App](#) →**Health Score****OPTIMAL****TEST PARAMETERS****What is Health Score ?**

Health Score is a comprehensive numerical indicator of an individual's overall health status. It converts complex medical data into a simple, easy-to-understand score that helps individuals quickly assess their current health and identify areas requiring attention.

Calculation Flow

P-LCC, BUN

These parameters are not part of your HealthScore calculation, as your HealthScore is calculated using selected parameters that reflect your overall wellness. Please consult your doctor if any of these values are flagged.



Ms Dr Namrata Shrivastava

Gender: **Female** Age: **51 Yrs** Patient ID: **16996029**

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DOCTOR CLINICAL OVERVIEW

BLOOD COUNT

Looks at blood cell counts and platelet parameters

Test Name	Result	Range
Hemoglobin	13.3 g/dL	12.0 - 15.0
TLC	8.1 $10^3/\mu\text{l}$	4 - 10
Neutrophils	61.9 %	40 - 80
Lymphocytes	28.7 %	20 - 40
Monocytes	5.3 %	2 - 10
Eosinophils	3.7 %	1 - 6
Basophils	0.4 %	0 - 2
Neutrophils.	5.01 $10^3/\mu\text{l}$	2 - 7
Lymphocytes.	2.32 $10^3/\mu\text{l}$	1 - 3
Monocytes.	0.43 $10^3/\mu\text{l}$	0.2 - 1.0
Eosinophils.	0.3 $10^3/\mu\text{l}$	0.02 - 0.5
Basophils.	0.03 $10^3/\mu\text{l}$	0.02 - 0.5
Platelet Count	300 $10^3/\mu\text{l}$	150 - 410
Mean Platelet Volume (MPV)	11.8 fL	9.3 - 12.1
PDW	15.1 fL	8.3 - 25
P-LCR	39.3 %	18 - 50
P-LCC	118 $10^9/\text{L}$	44 - 140

Out of Range Borderline Within Range



Ms Dr Namrata Shrivastava

Gender: Female Age: 51 Yrs Patient ID: 16996029

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DOCTOR CLINICAL OVERVIEW

ANEMIA STUDIES

Evaluates red blood cell indices for anemia assessment

Test Name	Result	Range
● RBC Count	4.7 $10^6/\mu\text{l}$	3.8 - 4.8
● PCV	41.6 %	36 - 46
● MCV	88.5 fl	83 - 101
● MCH	28.2 pg	27 - 32
● MCHC	31.9 g/dL	31.5 - 34.5
● RDW (CV)	14.6 %	11.6 - 14.0
● RDW-SD	46.4 fl	35.1 - 43.9
● Mentzer Index	18.83 %	—
● Red blood Cells	Absent /hpf	—

2 Out of Range 0 Borderline 7 Within Range

INFECTIOUS DISEASES

Markers indicating infection or immune response

Test Name	Result	Range
● PCT	0.4 %	0.17 - 0.32
● Deposit	Absent -	—
● Yeast Cells	Absent -	—
● Protozoa	Absent -	—

1 Out of Range 0 Borderline 3 Within Range

INFLAMMATION

Inflammation markers including ESR, CRP, fibrinogen and cytokines

Test Name	Result	Range
● ESR - Erythrocyte Sedimentation Rate	10 mm/hr	0 - 22

0 Out of Range 0 Borderline 1 Within Range



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DOCTOR CLINICAL OVERVIEW

SUGAR CONTROL

Blood glucose and HbA1c for diabetes assessment

Test Name	Result	Range
Glycosylated Hemoglobin (HbA1c)	5.4 %	0 - 5.7
Estimated Average Glucose	108.28 mg/dL	—
Glucose Fasting	108.1 mg/dL	70 - 100
Urine Glucose (sugar)	Negative -	—

Out of Range Borderline Within Range

LIVER FUNCTION

Liver enzymes, proteins and bilirubin levels

Test Name	Result	Range
Bilirubin Total	0.4 mg/dL	0.2 - 1.2
Bilirubin Direct	0.2 mg/dL	0 - 0.5
Bilirubin Indirect	0.2 mg/dL	0.1 - 1
SGOT/AST	11.7 U/L	5 - 34
SGPT/ALT	10.3 U/L	0 - 55
SGOT/SGPT Ratio	1.14 -	—
Alkaline Phosphatase	83 U/L	40 - 150
Total Protein	6.9 g/dL	6.4 - 8.3
Albumin	4.2 gm/dL	3.8 - 5
Globulin	2.7 g/dL	2.3 - 3.5
Albumin :Globulin Ratio	1.56 -	1 - 2.1
Gamma Glutamyl Transferase (GGT)	17.5 U/L	12 - 64
Bilirubin Urine	Negative -	—
Urobilinogen	Normal -	—

Out of Range Borderline Within Range



Ms Dr Namrata Shrivastava

Gender: **Female** Age: **51 Yrs** Patient ID: **16996029**

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DOCTOR CLINICAL OVERVIEW

KIDNEY FUNCTION

Kidney filtration markers and waste products

Test Name	Result	Range
Blood Urea	16.5 mg/dL	18 - 55
Bun	7.71 mg/dL	6 - 20
Creatinine	0.7 mg/dL	0.72 - 1.25
eGFR (CKD-EPI)	104.62 ml/min/1.73 sq m	—
Bun/Creatinine Ratio	11.01	12 - 20
Urea / Creatinine Ratio	23.57	25.68 - 42.8
Urine Protein (Albumin)	Negative -	—
Blood	Negative -	—
Crystals	Absent -	—
Cast	Absent -	—
Uric Acid	4.7 mg/dL	2.6 - 6.0
Calcium Serum	8.7 mg/dL	8.4 - 10.2

Out of Range Borderline Within Range



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DOCTOR CLINICAL OVERVIEW

URINALYSIS

Urinary physical and chemical examination parameters

Test Name	Result	Range
● Volume	20 mL	—
● Colour	Pale yellow -	—
● Transparency	Clear -	—
● Reaction (pH)	6.0 -	4.5 - 8.0
● Specific Gravity	1.010 -	1.010 - 1.030
● Urine Ketones (Acetone)	Negative -	—
● Nitrite	Negative -	—
● Epithelial Cells	3-5 /hpf	0 - 4
● Amorphous deposits	Absent -	—
● Bacteria	Occasional -	—
● Pus Cells (WBCs)	4-6 /hpf	0 - 5
● Leucocyte esterase	Negative -	—

0 Out of Range 0 Borderline 12 Within Range

HYDRATION MARKERS

Sodium, potassium, chloride balance

Test Name	Result	Range
● Phosphorus	4.1 mg/dL	2.3 - 4.7
● Sodium	142.8 mmol/L	136 - 145
● Potassium	4 mmol/L	3.5 - 5.1
● Chloride	104.1 mmol/L	98 - 107

0 Out of Range 0 Borderline 4 Within Range



Ms Dr Namrata Shrivastava

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DOCTOR CLINICAL OVERVIEW

CHOLESTEROL MARKERS

Cholesterol, cardiac enzymes and heart risk markers

Test Name	Result	Range
Total Cholesterol	192 mg/dL	0 - 200
Triglycerides	95.7 mg/dL	0 - 150
HDL Cholesterol	38.3 mg/dL	40 - 80
Non HDL Cholesterol	153.7 mg/dL	0 - 130
LDL Cholesterol	134.56 mg/dL	30 - 100
V.L.D.L Cholesterol	19.14 mg/dL	0 - 30
Chol/HDL Ratio	5.01 Ratio	3.5 - 5.0
HDL/ LDL Ratio	0.28 Ratio	0.5 - 3.0
LDL/HDL Ratio	3.51 Ratio	—

1 Out of Range 4 Borderline 4 Within Range

IRON LEVELS

Iron levels and binding capacity

Test Name	Result	Range
Iron	45 µg/dL	50 - 170

0 Out of Range 1 Borderline 0 Within Range

THYROID FUNCTION

Thyroid hormone balance

Test Name	Result	Range
Triiodothyronine (T3)	121.6 ng/dL	70 - 204
Total Thyroxine (T4)	8.5 µg/dL	5.0 - 12.5
Thyroid Stimulating Hormone (Ultrasensitive)	0.726 mIU/L	0.54 - 5.30

0 Out of Range 0 Borderline 3 Within Range

**Ms Dr Namrata Shrivastava**

Gender: Female Age: 51 Yrs Patient ID: 16996029

Your Health Summary

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COMPARATIVE HEALTH SUMMARY

Summary of Key Improvements / Declines

Outcomes

Total parameters improved

• Thyroid Stimulating Hormone (Ultrasensitive)

1**Still Out of range parameters detected**

No still out of range parameters detected

0**New Out of range parameters detected**

No new out of range parameters detected

0

PARAMETER WISE COMPARISON

Parameter	Current	Previous	Value Change	Current Status
Thyroid Stimulating Hormone (Ultrasensitive)	0.726 0.54-5.30 mIU/L 31-05-2026	11.14 0.54-5.30 mIU/L 25-01-2026	-10.4	Improved

[View your complete health trends and graphs in the app](#)

**Ms Dr Namrata Shrivastava**

Gender: Female Age: 51 Yrs Patient ID: 16996029

Your Health Summary

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PERSONALISED HEALTH SUMMARY

YOUR RESULTS INDICATE

Mildly Elevated Cholesterol Pattern

Your cholesterol pattern including LDL at 134.56 mg/dL and Non-HDL Cholesterol at 153.7 is mildly elevated. Small lifestyle changes — reducing fried food, more daily movement, and steady sleep — usually help bring these numbers back.

Healthy Liver Enzymes

Your liver enzymes ALT and AST are within the healthy range. These enzymes signal liver cell health — normal levels mean your liver is processing nutrients, clearing toxins, and supporting metabolism without stress.

Balanced Thyroid Markers

Your thyroid markers including TSH are within the healthy range. TSH signals your thyroid to produce hormones that control metabolism, heart rate, and body temperature — a balanced TSH means this regulatory loop is working smoothly.

HOW THIS MAY BE AFFECTING YOUR DAILY LIFE

What Mildly Elevated Cholesterol Means for You

Mildly elevated cholesterol usually has no direct symptoms, but your body is signaling a shift. Lighter meals, daily walks, and reducing fried food help keep cholesterol in check.

Your Liver is Supporting You Well

A healthy liver supports steady digestion, balanced energy after meals, and clear-headed focus. You are unlikely to notice bloating after fatty meals, unusual fatigue, or changes in skin tone — all quiet signs that your liver is processing smoothly.

Your Thyroid is Working Smoothly

A balanced thyroid keeps your metabolism, body temperature, and energy regulation consistent. You are less likely to notice unexplained weight shifts, feeling persistently cold, or unusual tiredness — all classic signs of thyroid imbalance that healthy levels protect you from.



Ms Dr Namrata Shrivastava

Gender: Female Age: 51 Yrs Patient ID: 16996029

Your Health Summary

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YOUR PERSONAL HEALTH ROADMAP

This Week

Do these first

1. Discuss your lipid numbers with your doctor at your next visit.
2. Book your free Redcliffe report consultation to understand your lipid pattern.
3. Reduce fried food this week and add a 20-minute daily walk.
4. Track how you feel after meals, especially heavy or oily ones.

In the Next Month

Build momentum

1. Build a steady walking or light exercise routine through the month.
2. Follow up with your doctor next month to review your lipid numbers.
3. Note any changes in your energy and stamina through the month.
4. Plan a lipid panel retest as your doctor advises.

In 2-3 Months

Track your progress

1. Retest the lipid panel at the 2-3 month mark to track your progress.
2. Review the 2-3 month lipid results with your doctor.
3. Schedule your Redcliffe Heart Health package to monitor cholesterol.
4. Maintain balanced meals and regular movement as a long-term habit.



Smart Reports



AI Assistance



Report Consultation



Diet Plan



CARE by
Redcliffe Labs

Scan QR code

Patient NAME : Ms Dr Namrata Shrivastava

DOB/Age/Gender : 51 Y/Female

Patient ID / UHID : 16996029/RCL9419054

Referred BY : Self

Sample Collected : May 31, 2026, 06:16 AM

Report STATUS : Final Report

Barcode NO : 33339415

Sample Type : Whole blood EDTA

Report Date : May 31, 2026, 12:18 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

Fit India Full Body Checkup with Free HbA1c**Complete Blood Count (CBC)**

RBC Parameters			
Hemoglobin <i>colorimetric</i>	13.3	g/dL	12.0 - 15.0
RBC Count <i>Electrical impedance</i>	4.7	10 ⁶ /μl	3.8 - 4.8
PCV <i>Calculated</i>	41.6	%	36 - 46
MCV <i>Calculated</i>	88.5	fl	83 - 101
MCH <i>Calculated</i>	28.2	pg	27 - 32
MCHC <i>Calculated</i>	31.9	g/dL	31.5 - 34.5
RDW (CV) * <i>Calculated</i>	14.6 H*	%	11.6 - 14.0
RDW-SD * <i>Calculated</i>	46.4 H*	fl	35.1 - 43.9
WBC Parameters			
TLC <i>Electrical impedance and microscopy</i>	8.1	10 ³ /μl	4 - 10
Differential Leucocyte Count			
Neutrophils	61.9	%	40-80
Lymphocytes	28.7	%	20-40
Monocytes	5.3	%	2-10
Eosinophils	3.7	%	1-6
Basophils	0.4	%	<2
Absolute Leukocyte Counts <i>calculated</i>			
Neutrophils.	5.01	10 ³ /μl	2 - 7
Lymphocytes.	2.32	10 ³ /μl	1 - 3
Monocytes.	0.43	10 ³ /μl	0.2 - 1.0
Eosinophils.	0.3	10 ³ /μl	0.02 - 0.5
Basophils.	0.03	10 ³ /μl	0.02 - 0.5
Platelet Parameters			
Platelet Count <i>Electrical impedance and microscopy</i>	300	10 ³ /μl	150 - 410
Mean Platelet Volume (MPV) * <i>Calculated</i>	11.8	fL	9.3 - 12.1
PCT * <i>Calculated</i>	0.4 H*	%	0.17 - 0.32

Note :- (H* - High, L* - Low, CL* - Critical Low, CH* - Critical High)

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.


 Dr. Manju Saraf
 MBBS, DCP
 Consultant Pathologist


Booking Centre :- Home Collection

Processing Lab :- Redcliffe Lifetech Pvt. Ltd., New 639, old 295, Napier, 1st Floor Town opp. Bhanwartal, Garden, Jabalpur. 482001

📞 898-898-0606

✉ care@redcliffelabs.com

🌐 www.redcliffelabs.com

All Lab results are subject to clinical interpretation by qualified medical professional and this report is not subject to use for any medico-legal purpose.

Patient NAME : Ms Dr Namrata Shrivastava

DOB/Age/Gender : 51 Y/Female

Patient ID / UHID : 16996029/RCL9419054

Referred BY : Self

Sample Collected : May 31, 2026, 06:16 AM

Report STATUS : Final Report

Barcode NO : 33339415

Sample Type : Whole blood EDTA

Report Date : May 31, 2026, 12:18 PM.



Test Description	Value(s)	Unit(s)	Reference Range
PDW * <i>Calculated</i>	15.1	fL	8.3 - 25.0
P-LCR * <i>Calculated</i>	39.3	%	18 - 50
P-LCC * <i>Calculated</i>	118	10 ⁹ /L	44 - 140
Mentzer Index * <i>Calculated</i>	18.83	%	> 13

Interpretation:

CBC provides information about red cells, white cells and platelets. Results are useful in the diagnosis of anemia, infections, leukemias, clotting disorders and many other medical conditions.

Note :- (H* - High , L* - Low ,CL* - Critical Low,CH* - Critical High)

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Patient NAME : Ms Dr Namrata Shrivastava

DOB/Age/Gender : 51 Y/Female

Patient ID / UHID : 16996029/RCL9419054

Referred BY : Self

Sample Collected : May 31, 2026, 06:16 AM

Report STATUS : Final Report

Barcode NO : 33339415

Sample Type : Whole blood EDTA

Report Date : May 31, 2026, 02:10 PM.

Test Description	Value(s)	Unit(s)	Reference Range
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Erythrocyte Sedimentation Rate (ESR)

ESR - Erythrocyte Sedimentation Rate <i>Modified Westergren</i>	10	mm/hr	0-22
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Interpretation:

ESR is also known as Erythrocyte Sedimentation Rate. An ESR test is used to assess inflammation in the body. Many conditions can cause an abnormal ESR, so an ESR test is typically used with other tests to diagnose and monitor different diseases. An elevated ESR may occur in inflammatory conditions including infection, rheumatoid arthritis, systemic vasculitis, anemia, multiple myeloma, etc. Low levels are typically seen in congestive heart failure, polycythemia, sickle cell anemia, hypo fibrinogenemia, etc.

Reference- Dacie and Lewis practical hematology

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Patient NAME : Ms Dr Namrata Shrivastava

DOB/Age/Gender : 51 Y/Female

Patient ID / UHID : 16996029/RCL9419054

Referred BY : Self

Sample Collected : May 31, 2026, 06:16 AM

Report STATUS : Final Report

Barcode NO : 33339415

Sample Type : Whole blood EDTA

Report Date : May 31, 2026, 01:55 PM.



Test Description	Value(s)	Unit(s)	Reference Range
------------------	----------	---------	-----------------

HbA1C (Glycosylated Haemoglobin)

Glycosylated Hemoglobin (HbA1c) <i>Immunoturbidimetry</i>	5.4	%	< 5.7
Estimated Average Glucose *	108.28	mg/dL	Refer Table Below

Interpretation:**Interpretation For HbA1c% As per American Diabetes Association (ADA)**

Reference Group	HbA1c in %
Non diabetic adults ≥ 18 years	<5.7
At risk (Prediabetes)	5.7 - 6.4
Diagnosing Diabetes	≥ 6.5
Therapeutic goals for glycemic control	Age > 19 years Goal of therapy: < 7.0 Age < 19 years Goal of therapy: <7.5

Note:

1. Since HbA1c reflects long term fluctuations in the blood glucose concentration, a diabetic patient who is recently under good control may still have a high concentration of HbA1c. Converse is true for a diabetic previously under good control but now poorly controlled.
2. Target goals of < 7.0 % may be beneficial in patients with short duration of diabetes, long life expectancy and no significant cardiovascular disease. In patients with significant complications of diabetes, limited life expectancy or extensive co-morbid conditions, targeting a goal of < 7.0 % may not be appropriate.

Comments :

HbA1c provides an index of average blood glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations ADA criteria for correlation between HbA1c & Mean plasma glucose levels.

HbA1c(%)	Mean Plasma Glucose (mg/dL)	HbA1c(%)	Mean Plasma Glucose (mg/dL)
6	126	12	298
8	183	14	355
10	240	16	413

(*) Parameter(s) are outside the scope of tests recognized under the NABL M(EL)T Scheme.


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 Consultant Pathologist


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Patient NAME : Ms Dr Namrata Shrivastava

DOB/Age/Gender : 51 Y/Female

Patient ID / UHID : 16996029/RCL9419054

Referred BY : Self

Sample Collected : May 31, 2026, 06:16 AM

Report STATUS : Final Report

Barcode NO : 32488011

Sample Type : FLUORIDE F

Report Date : May 31, 2026, 12:23 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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Blood Sugar Fasting

Glucose Fasting GOD-POD/HK G8P-DH	108.1 H*	mg/dL	70 - 100
--------------------------------------	----------	-------	----------

Interpretation:

Status	Fasting plasma glucose in mg/dL
Normal	70 - 100
Impaired fasting glucose	101 - 125
Diabetes	≥126

Reference : American Diabetes Association**Comment :**

Blood glucose determinations are commonly used as an aid in the diagnosis and treatment of diabetes. Elevated glucose levels (hyperglycemia) may also occur with pancreatic neoplasm, hyperthyroidism, and adrenal cortical hyper function as well as other disorders. Decreased glucose levels (hypoglycemia) may result from excessive insulin therapy, insulinoma, or various liver diseases.

Note

1. The diagnosis of Diabetes requires a fasting plasma glucose of $\geq$ 126 mg/dL or a random / 2 hour plasma glucose value of $\geq$ 200 mg/dL with symptoms of diabetes mellitus.
2. Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis.

Note :- (H* - High, L* - Low, CL* - Critical Low, CH* - Critical High)

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Patient NAME : Ms Dr Namrata Shrivastava

DOB/Age/Gender : 51 Y/Female

Patient ID / UHID : 16996029/RCL9419054

Referred BY : Self

Sample Collected : May 31, 2026, 06:16 AM

Report STATUS : Final Report

Barcode NO : 33320361

Sample Type : Serum

Report Date : May 31, 2026, 03:32 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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Liver Function Test (LFT)

Bilirubin Total <i>Diazonium Ion</i>	0.4	mg/dL	0.2 - 1.2
Bilirubin Direct * <i>Diazo Reaction</i>	0.2	mg/dL	0.0 - 0.5
Bilirubin Indirect * <i>Calculation (T Bil - D Bil)</i>	0.2	mg/dL	0.1 - 1.0
SGOT/AST <i>IFCC without P5P</i>	11.7	U/L	5 - 34
SGPT/ALT <i>IFCC without P5P</i>	10.3	U/L	0 to 55
SGOT/SGPT Ratio *	1.14	-	-
Alkaline Phosphatase <i>p-nitrophenyl Phosphate, AMP buffer</i>	83	U/L	40 - 150
Total Protein <i>Biuret</i>	6.9	g/dL	6.4 - 8.3
Albumin <i>BCG</i>	4.2	gm/dL	3.8 - 5.0
Globulin * <i>Calculation (T.P - Albumin)</i>	2.7	g/dL	2.3 - 3.5
Albumin :Globulin Ratio * <i>Calculation (Albumin/Globulin)</i>	1.56	-	1.0 - 2.1
Gamma Glutamyl Transferase (GGT) * <i>ENZYMATIC</i>	17.5	U/L	12 - 64

Interpretation:

The liver filters blood, metabolizes nutrients, detoxifies harmful substances, and produces blood clotting proteins. Liver cells contain enzymes that facilitate these functions. When cells are damaged, enzymes leak into the blood, detectable through blood tests.

Key enzymes tested:

- 1. AST (SGOT):** may indicate tissue injury / damage in muscles or liver.
- 2. ALT (SGPT):** Primarily in the liver. Elevated ALT and AST suggest liver damage.
- 3. Alkaline Phosphatase & GGT:** Linked to bile production and flow. Elevated levels may indicate bile flow issues related to the liver, gallbladder, or bile ducts.

Blood proteins, **albumin and globulin**, are essential for growth, development, and health.

- 1. Low protein:** May indicate bleeding, liver disorders, malnutrition, or agammaglobulinemia.
- 2. High protein (Hyperproteinemia):** Often due to dehydration or increased protein production.
- 3. Low albumin:** Caused by poor diet, kidney, or liver disease.
- 4. High albumin:** Usually due to severe dehydration.

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Patient NAME : Ms Dr Namrata Shrivastava

DOB/Age/Gender : 51 Y/Female

Patient ID / UHID : 16996029/RCL9419054

Referred BY : Self

Sample Collected : May 31, 2026, 06:16 AM

Report STATUS : Final Report

Barcode NO : 33320361

Sample Type : Serum

Report Date : May 31, 2026, 03:32 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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Kidney Function Test (KFT)

Blood Urea <i>Urease-GLDH</i>	16.5 L*	mg/dL	18 - 55
Bun * <i>Urease</i>	7.71	mg/dL	6 - 20
Creatinine <i>Modified Jaffe</i>	0.7 L*	mg/dL	0.72 - 1.25
eGFR (CKD-EPI)	104.62	ml/min/1.73 sq m	Normal Or High: >= 90 Mild Or Decrease: 60-89 Mild To Moderate Decrease: 45-59 Mild To Severe Decrease: 30-44 Severe Decrease: 15-29 Kidney Failure: < 15
Bun/Creatinine Ratio * <i>Calculated</i>	11.01 L*		12 - 20
Urea / Creatinine Ratio * <i>Calculated</i>	23.57 L*		25.68- 42.8
Uric Acid <i>Uricase</i>	4.7	mg/dL	2.6 - 6.0
Calcium Serum <i>Arsenazo III</i>	8.7	mg/dL	8.4 - 10.2
Phosphorus <i>UV Molybdate</i>	4.1	mg/dL	2.3 - 4.7
Sodium <i>Direct ISE</i>	142.8	mmol/L	136 - 145
Potassium <i>Direct ISE</i>	4	mmol/L	3.5 - 5.1
Chloride <i>Direct ISE</i>	104.1	mmol/L	98 - 107

Interpretation:

Kidney function tests is a collective term for a variety of individual tests and procedures that can be done to evaluate how well the kidneys are functioning. Many conditions can affect the ability of the kidneys to carry out their vital functions. Some lead to a rapid (acute) decline in kidney function others lead to a gradual (chronic) decline in function. Both result in a buildup of toxic waste substances done on urine samples, as well as on blood samples. A number of symptoms may indicate a problem with your kidneys. These include : high blood pressure, blood in urine, frequent urges to urinate, difficulty beginning urination, painful urination, swelling in the hands and feet due to a buildup of fluids in the body. A single symptom may not mean something serious. However, when occurring simultaneously, these symptoms suggest that your kidneys are not working properly. Kidney function tests can help determine the reason. Ionized calcium this test if you have signs of kidney or parathyroid disease. The test may also be done to monitor progress and treatment of these diseases. "eGFR test is applicable for patients aged 18 years or more."

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Test Description	Value(s)	Unit(s)	Reference Range
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Lipid Profile

Total Cholesterol <i>CHOD-PAP</i>	192	mg/dL	<200
Triglycerides <i>Method: GPO</i>	95.7	mg/dL	<150
HDL Cholesterol <i>Direct Measure PEG</i>	38.3 L*	mg/dL	>40
Non HDL Cholesterol * <i>Calculated</i>	153.7 H*	mg/dL	<130
LDL Cholesterol * <i>Calculated</i>	134.56 H*	mg/dL	<100
V.L.D.L Cholesterol * <i>Calculated</i>	19.14	mg/dL	< 30
Chol/HDL Ratio * <i>Calculated</i>	5.01 H*	Ratio	3.5 - 5.0
HDL/ LDL Ratio * <i>Calculated</i>	0.28 L*	Ratio	0.5 - 3.0
LDL/HDL Ratio * <i>Calculated</i>	3.51	Ratio	-

Interpretation:

Lipid level assessments must be made following 10 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. NCEP recommends of 3 different samples to be drawn at intervals of 1 week for harmonizing biological variables that might be encountered in single assays. Intraindividual (within-person) variation in triglyceride (TG) levels is high, often showing a 12.9% to 40.8% variation in healthy individuals within 1 year. This variability is driven by a combination of biological, lifestyle, and physiological factors that fluctuate rather than remaining constant.

Causes of variation include

- Diet: Levels spike 5–10 times after meals. Unhealthy high saturated fat diets like non veg diet sweetened beverages, fried or processed foods, and alcohol intake can significantly raise triglyceride values.
- Lifestyle: Obesity, sedentary lifestyle, and smoking can raise levels.
- Biology: Pregnancy (estrogen), aging, and conditions like Diabetes or Metabolic Syndrome cause major shifts.
- Drugs like: Beta-blockers, steroids, tamoxifen, thiazides, and oral contraceptives can raise triglyceride levels

National Lipid Association Recommendations (NLA-2014)	Total Cholesterol (mg/dL)	Triglyceride (mg/dL)	LDL Cholesterol (mg/dL)	Non HDL Cholesterol (mg/dL)
Optimal	<200	<150	<100	<130
Above Optimal			100-129	130 - 159
Borderline High	200-239	150-199	130-159	160 - 189
High	>=240	200-499	160-189	190 - 219
Very High	-	>=500	>=190	>=220

HDL Cholesterol	
Low	High
<40	>=60

Risk Stratification for ASCVD (Atherosclerotic Cardiovascular Disease) by Lipid Association of India.

Note :- (H* - High, L* - Low, CL* - Critical Low, CH* - Critical High)

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Barcode NO : 33320361

Sample Type : Serum

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Test Description		Value(s)	Unit(s)	Reference Range
Risk Category	A. CAD with > 1 feature of high risk group			
Extreme risk group	B. CAD with >1 feature of very high risk group of recurrent ACS (within 1 year) despite LDL-C <or = 50 mg/dl or poly vascular disease			
Very High Risk	1.Established ASCVD 2.Diabetes with 2 major risk factors of evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia			
High Risk	1. Three major ASCVD risk factors 2. Diabetes with 1 major risk factor or no evidence of end organ damage 3. CHD stage 3B or 4. 4 LDL >190 mg/dl 5. Extreme of a single risk factor 6. Coronary Artery Calcium - CAC > 300 AU 7. Lipoprotein a >= 50 mg/dl 8. Non stenotic carotid plaque			
Moderate Risk	2 major ASCVD risk factors			
Low Risk	0-1 major ASCVD risk factors			
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors				
1. Age >=45 years in Males & >= 55 years in Females	3. Current Cigarette smoking or tobacco use			
2. Family history of premature ASCVD	4. High blood pressure			
5. Low HDL				

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by Lipid Association of India in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal <OR = 30)	<80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	>OR = 30	>OR = 60	> 30	> 60
Very High Risk	<50	<80	>OR = 50	>OR = 80
High Risk	<70	<100	>OR = 70	>OR = 100
Moderate Risk	<100	<130	>OR = 100	>OR = 130
Low Risk	<100	<130	>OR = 130*	>OR = 160

* After an adequate non-pharmacological intervention for at least 3 months.

References :

- Management of Dyslipidaemia for the Prevention of Stroke : Clinical practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.
- Nordestgaard, B. A Test in Context: Lipid Profile, Fasting Versus Nonfasting. JACC. 2017 Sep, 70 (13) 1637-1646.
- P.N.M. Demacker, R.W.B. Schade, R.T.P. Jansen, A. Van & Laar, Intra-individual variation of serum cholesterol, triglycerides and high density lipoprotein cholesterol in normal humans, Atherosclerosis.
- Parhofer KG, Laufs U: The diagnosis and treatment of hypertriglyceridemia. Dtsch Arztebl Int 2019; 116: 825-32. DOI: 10.3238/arztebl.2019.0825

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Barcode NO : 33320361

Referred BY : Self

Sample Type : Serum

Sample Collected : May 31, 2026, 06:16 AM

Report Date : May 31, 2026, 02:08 PM.

Test Description	Value(s)	Unit(s)	Reference Range
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Thyroid Profile Total

Triiodothyronine (T3) ECLIA	121.6	ng/dL	70 - 204
Total Thyroxine (T4) ECLIA	8.5	µg/dL	5.0- 12.5
Thyroid Stimulating Hormone (Ultrasensitive) ECLIA	0.726	mIU/L	0.54 - 5.30

Interpretation:

Pregnancy	Reference Range TSH
1st Trimester	0.1 - 2.5
2nd Trimester	0.2 - 3.0
3rd Trimester	0.3 - 3.0

Note: TSH levels are subject to circadian variation, reaching peak levels between 2-4 am. and at a minimum between 6-10 pm. The variation is of 50 %, hence time of the day has influence on the measured serum TSH concentrations. Apart from nocturnal circadian surge other biological factors responsible for TSH variation include pulsatile secretion, along with seasonal changes and lifestyle factors such as smoking, BMI, and diet. Other contributors include aging, non-thyroidal illnesses, medication interference, and rare analytical issues like macro-TSH

Clinical Use:

1. Diagnose Hypothyroidism & Hyperthyroidism
2. Monitor T4 therapy
3. Measure subnormal TSH levels

Increased TSH: Primary hypothyroidism, Subclinical hypothyroidism, TSH-dependent hyperthyroidism, Thyroid hormone resistance

Decreased TSH: Graves' disease, Autonomous thyroid hormone secretion, TSH deficiency

Thyroid malfunction (hyper or hypo) affects T3 & T4 levels. Pituitary or hypothalamic issues also influence thyroid activity.

1. Primary Hypothyroidism: High TSH levels.

2. Secondary/Tertiary Hypothyroidism: Low TSH levels.

3. Euthyroid Sick Syndrome: Abnormal thyroid test results due to non-thyroidal illnesses (NTI).

TBG levels are stable in healthy individuals but may be altered by pregnancy, estrogens, androgens, steroids, or glucocorticoids, causing inaccurate T3 & T4 readings.

TSH	T4	T3	Interpretation
High	Normal	Normal	Mild (subclinical) hypothyroidism
High	Low	Low Or Normal	Hypothyroidism
Low	Normal	Normal	Mild (subclinical) hyperthyroidism
Low	High Or Normal	High Or Normal	Hyperthyroidism
Low	Low Or Normal	Low Or Normal	Nonthyroidal illness; pituitary (secondary) hypothyroidism
Normal	High	High	Thyroid hormone resistance syndrome (a mutation in the thyroid hormone receptor decreases thyroid hormone function)

References

- Van der Spoel E, Roelfsema F and van Heemst D (2021) Within-Person Variation in Serum Thyrotropin Concentrations: Main Sources, Potential Underlying Biological Mechanisms, and Clinical Implications. Front. Endocrinol. 12:619568. doi: 10.3389/fendo.2021.619568

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Sample Type : Serum

Report Date : May 31, 2026, 03:32 PM.

Test Description	Value(s)	Unit(s)	Reference Range
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Iron

Iron <i>Ferene</i>	45 L*	µg/dL	50 - 170
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Interpretation:

Iron is body's essential trace element used for differential diagnosis of anemias, diagnosis of hemochromatosis and hemosiderosis.

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Patient ID / UHID : 16996029/RCL9419054

Referred BY : Self

Sample Collected : May 31, 2026, 06:19 AM

Report STATUS : Final Report

Barcode NO : 32488060

Sample Type : Spot Urine

Report Date : May 31, 2026, 01:38 PM.



Test Description	Value(s)	Unit(s)	Reference Range
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Urine Routine and Microscopic Examination

Physical Examination *			
Volume *	20	mL	-
Colour *	Pale yellow	-	Pale yellow
Transparency *	Clear	-	Clear
Deposit *	Absent	-	Absent
Chemical Examination *			
Reaction (pH) <i>Double Indicator</i>	6.0	-	4.5 - 8.0
Specific Gravity <i>Ion Exchange</i>	1.010	-	1.010 - 1.030
Urine Glucose (sugar) <i>Oxidase / Peroxidase</i>	Negative	-	Negative
Urine Protein (Albumin) <i>Acid / Base Colour Exchahnge</i>	Negative	-	Negative
Urine Ketones (Acetone) <i>Legals Test</i>	Negative	-	Negative
Blood <i>Peroxidase Hemoglobin</i>	Negative	-	Negative
Leucocyte esterase <i>Enzymatic Reaction</i>	Negative	-	Negative
Bilirubin Urine	Negative	-	Negative
Nitrite <i>Griless Test</i>	Negative	-	Negative
Urobilinogen <i>Ehrlichs Test</i>	Normal	-	Normal
Microscopic Examination *			
Pus Cells (WBCs) *	4-6	/hpf	0 - 5
Epithelial Cells *	3-5	/hpf	0 - 4
Red blood Cells *	Absent	/hpf	Absent
Crystals *	Absent	-	Absent
Cast *	Absent	-	Absent
Yeast Cells *	Absent	-	Absent
Amorphous deposits *	Absent	-	Absent
Bacteria *	Occasional	-	Absent
Protozoa *	Absent	-	Absent

Interpretation:

URINALYSIS- Routine urine analysis assists in screening and diagnosis of various metabolic, urological, kidney and liver disorders.

Protein: Elevated proteins can be an early sign of kidney disease. Urinary protein excretion can also be temporarily elevated by strenuous exercise, orthostatic proteinuria, dehydration, urinary tract infections and acute illness with fever

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Test Description	Value(s)	Unit(s)	Reference Range
Glucose: Uncontrolled diabetes mellitus can lead to presence of glucose in urine. Other causes include pregnancy, hormonal disturbances, liver disease and certain medications. Ketones: Uncontrolled diabetes mellitus can lead to presence of ketones in urine. Ketones can also be seen in starvation, frequent vomiting, pregnancy and strenuous exercise. Blood: Occult blood can occur in urine as intact erythrocytes or haemoglobin, which can occur in various urological, nephrological and bleeding disorders. Leukocytes: An increase in leukocytes is an indication of inflammation in urinary tract or kidneys. Most common cause is bacterial urinary tract infection. Nitrite: Many bacteria give positive results when their number is high. Nitrite concentration during infection increases with length of time the urine specimen is retained in bladder prior to collection. pH: The kidneys play an important role in maintaining acid base balance of the body. Conditions of the body producing acidosis/ alkalosis or ingestion of certain type of food can affect the pH of urine. Specific gravity: Specific gravity gives an indication of how concentrated the urine is. Increased specific gravity is seen in conditions like dehydration, glycosuria and proteinuria while decreased specific gravity is seen in excessive fluid intake, renal failure and diabetes insipidus. Bilirubin: In certain liver diseases such as biliary obstruction or hepatitis, bilirubin gets excreted in urine. Urobilinogen: Positive results are seen in liver diseases like hepatitis and cirrhosis and in cases of haemolytic anaemia.			

*** End Of Report ***

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6. All turnaround times are indicative, not guaranteed. Delays may occur due to test complexity, sample condition, referral testing, or quality control procedures, logistics, or other operational constraints.
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11. In case of critical, alarming, or unexpected test results, you are advised to contact the laboratory immediately for further guidance and thereafter consult a qualified medical practitioner without delay. Test results should always be interpreted in conjunction with the patient's clinical history and clinical findings.
12. Accessing reports through digital platforms constitutes acceptance of electronic delivery and record storage.
13. The laboratory shall not be liable for any delays in performance or failure to perform its obligations arising out of or in connection with events beyond reasonable control, including but not limited to natural disasters, pandemics, acts of God, strikes, labor disputes, transportation disruptions, regulatory or governmental actions, technical failures, or any other unforeseen circumstances.
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